

NHS Continuing Healthcare Frequently Asked Questions

Q1: The Checklist should be used when there may be a need for CHC and the individual's ongoing needs are known. When is it NOT appropriate to screen for CHC?

A: Checklist may not be appropriate if:

- It is clear to practitioners that there is no need for CHC at this time
- The individual has short term health needs and has not returned to their optimum condition
- It has been agreed to proceed directly with a DST
- The individual has a rapidly deteriorating condition and may be entering a terminal phase (use Fast Track Pathway instead)
- Individual's needs are fully met by Section 117 of the Mental Health Act (MHA)

Q2: Should the CHC assessment be used to support discharge from hospital?

A: The National Framework is clear that for individuals in hospital CHC assessment should not happen until their condition has stabilised and their long term needs are clear. In most cases this will be after they are discharged.

The majority of individuals will not have a full assessment for CHC while they are in hospital.

Q3: Does a positive Checklist mean that an individual is likely to be eligible for CHC?

A: No. The Checklist threshold has been set intentionally low to ensure that those that may need a full assessment for CHC receive it.

In practice, the majority of individuals with a positive Checklist are not eligible for CHC

Q4: What are the requirements for an effective MDT meeting?

A:

- Expectations are not raised before the meeting
- Everyone has prepared for the meeting
- An effective Coordinator and someone taking notes
- Everyone understands their role and contribution
- Everyone has the opportunity to share their views
- Everyone behaves appropriately
- Clear decisions are made
- Disagreements resolved with evidence based discussion

Q5: After receiving a positive checklist, how long should it take to make a decision about whether an individual is eligible for CHC?

A: The National Framework expects CCGs to make a decision about eligibility within 28 days from when they receive a completed Checklist. This includes completing the assessment and verification processes.

Q6: Can you establish whether an individual has a Primary Health Need by counting up the levels of need in each domain of the DST?

A: No. Needs have to be considered in line with the four key characteristics; nature, intensity, complexity and unpredictability, to determine if an individual has a primary health need. However, the tool sets out an indicative guideline that one priority or two severe domains will usually indicate a primary health need.

Q7: What is the role of Local Authorities in the CHC process?

A: Local Authorities provide advice and assistance to a CCG for individual cases:

- Providing staff for multi-disciplinary teams (paragraph 28 of NF)
- Responding to requests for information from CCGs
- Implementation of the Care Act

Q8: What is the role of the referrer in the CHC pathway?

A: The referrer is often the first professional an individual speaks to about CHC.

They have a responsibility to set realistic expectations and should be clear that the process is about assessing need rather than a financial exercise.

They should discuss the low conversion rate for eligibility and what options for meeting their need may be available if they are found eligible..

Q9: What are the characteristics of a good referral?

A: A good quality referral ensures that the process is smooth and effective for the individual.

In order for a referral to be processed it should meet these criteria:

- Include appropriate evidence
- Has been discussed with the individual or representative and completed with them, if possible
- Either a copy of the Public Information leaflet or the link to access the leaflet on line, has been given to the individual
- Consent gained from the individual or Best Interest assessment with representatives of the individual if they lack capacity.

Q10: Who can complete a Checklist?

A: The National Framework states that any health or social care practitioner that has had the relevant training may complete a Checklist. Staff who require training are asked to contact the CHC Lead within their organisation (see contacts at end of this FAQ sheet). The CCGs do not accept Checklists from care homes.

Q11: What do I do if a person currently in receipt of funded nursing care, or a joint funded package of care, has a change in needs which might indicate CHC eligibility?

A: Complete a Checklist and submit it to the CHC service

Q12: What do I need to do after I have completed the Checklist?

A: All checklists, whatever the outcome (ie positive or negative) must be recorded on your internal systems and immediately sent to the CHC Team who will organise a MDT to carry out a CHC

assessment where the Checklist is positive. Where the Checklist is negative, the CHC Team will send a letter to the individual advising them of the outcome of the Checklist and advising how they can appeal if they are unhappy with it.

Q13: How much notice will I be given to attend a MDT meeting following submission of the Checklist?

A: Please give your availability to attend the MDT meeting in the section at the back of the Checklist. The CCGs are required to ensure that the majority (over 80%) of CHC assessments are completed and decisions confirmed within 28 days of receipt of the Checklist. Therefore the CHC service will need to organise the MDT meeting to take place no later than day 21 in order to allow sufficient time for the Decision Support Tool to be completed and submitted to the CCG for verification of the MDT's recommendation.

Q14: Can the CHC Team give social care more notice when arranging a review?

A: Only new referrals (ie Checklists) require a CCG decision within 28 days. Therefore health and social care practitioners have more flexibility to arrange a review date for existing care packages that are CHC or joint funded, and should work together to agree a mutually convenient date with the individual and family. It should be noted that the revised National Framework stipulates that reviews for CHC eligible individuals should focus on the care package and a re-assessment of eligibility should not be undertaken unless there is a clear change in need which indicates a change in eligibility. In addition, a local agreement has been made by all CCG and social care partners across Greater and Mid-Notts which states that individuals in receipt of joint funded (including funded nursing care) packages will only have a full MDT review when there is a material change in need. Therefore the number of requests for a social care practitioner to attend a MDT will reduce.

Q15: What about Fast Track reviews?

A: The CCGs require all individuals in receipt of a fast track CHC care package to be reviewed at 8 weeks. Therefore requests for a social care practitioner to attend MDTs for fast track reviews will continue.

Q16: Can the CHC Team share the latest Decision Support Tool when a request is made for a social care practitioner to attend a review for a person who is currently in receipt of CHC?

A: Yes, social care practitioners are asked to request a copy of the latest DST if this is not already held by them.

Q17: Should a Checklist be completed for every case presented to a Social Care Panel for funding?

A: No – see question 1 above.

Contacts

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